National Cancer Control Programme (NCCP) 00378 · clinician draft

CMF · intravenous (IV) · 28-day schedule

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · CMF · intravenous (IV) · 28-day schedule

Active medicines: Cyclophosphamide + Methotrexate + Fluorouracil (5-FU)

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before chemotherapy on Day 1 Review QT risk, constipation, hepatic function and any alternative local regimen.
Dexamethasone	8 mg orally once daily	Days 1, 2 and 3 Reconcile total steroid exposure; individual adjustments require prescribing review.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be specified Review renal function, contraindications and interactions before prescribing.

Additional phase-specific support

- Confirm allergy/interaction review, blood tests and the source-specific renal/hepatic assessment before prescribing.
- Confirm dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) status before first fluorouracil and review partial deficiency with the consultant.
- Confirm an individual hydration and bladder-symptom plan; adapt fluid advice to cardiac/renal restrictions.
- Review pleural effusion or ascites because methotrexate can accumulate in third-space fluid.
- Confirm the dose is going ahead before starting the linked support course. Reconcile rescue medicine duration rather than copying another treatment day.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00378 v6 · p3 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.18 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 8 · CMF · intravenous (IV) · 28-day schedule

Active medicines: Cyclophosphamide + Methotrexate + Fluorouracil (5-FU)

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before chemotherapy on Day 8 Review QT risk, constipation, hepatic function and any alternative local regimen.
Dexamethasone	8 mg orally once daily	Days 8, 9 and 10 Reconcile total steroid exposure; individual adjustments require prescribing review.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be specified Review renal function, contraindications and interactions before prescribing.

Additional phase-specific support

- Confirm allergy/interaction review, blood tests and the source-specific renal/hepatic assessment before prescribing.
- Confirm dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) status before first fluorouracil and review partial deficiency with the consultant.
- Confirm an individual hydration and bladder-symptom plan; adapt fluid advice to cardiac/renal restrictions.
- Review pleural effusion or ascites because methotrexate can accumulate in third-space fluid.
- Confirm the dose is going ahead before starting the linked support course. Reconcile rescue medicine duration rather than copying another treatment day.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-10-25 · SACTCheck v0.84.18 · Regimen-level drafts; prescriber and oncology-pharmacy verification required